N M Wadia Institute Of Cardiology

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Discharge Summary

Patient Name	: Mr. Radhe Shyam Mittal	**Age/Sex**	: 58 Yrs/Male
PRN	: 113258	**Visit**	: IP2
Clinician Incharge	: Dr. D Gaware	**Admission Date**	: 18/04/2019 14:39
Referred. By 1	:	**Discharge Date**	: 20/04/2019 17:53:00
Department	: Cardiology	**Referred. By 2**	:
MLC No.	:		

Allergic To

NOT KNOWN

Blood Group :

Diagnosis

Provisional : GIDDINESS UNDER EVALUATION

Final : DIABETES MELITUS[E11.9]
HYPERTENSION [I10]
ISCHEMIC HEART DISEASE [I25]
POST CABG STATUS [I25.810]
LV DYSFUNCTION
A/W HYPOGLYCEMIA
VENTRICULAR ECTOPICS-BIGEMINY,TRIGEMINY [I49.3]

Chief Complaints on admission

58 YEARS MALE K/C/O DM/ HTN/IHD.POST CABG STATUS, ADMITTED WITH H/O SUDDEN GIDDINESS AT 12/30 PM AT HOME ON18/4/19, BSL CHECKED AT HOME73MG/DL, BP WAS 110/70. CONSULT WITH DR GAWARE AND ADMITTED FOR FURTHER EVALUATION IN NMWH.

Medical/Surgical History

DIABETES MELITUS SINCE 28/11/18
HYPERTENSION SINCE 28/11/18
POST CABG STATUS(3/4/19)

Clinical findings on admission

G.C :MODERATE

Temprature	98° F	**Weight**	(kg)	**Pallor**	1+
Pulse Rate	88/Min	**Height**	(cm)	**Icterus**	NO
R.R.	18/Min	**BMI**	(kg/m2)	**Cyanosis**	NO
B.P.	100/80 mm of Hg			**Oedema**	NO

P/A :SOFT NONTEDER

CVS :S1S2+

RS :CLEAR

CNS :CONSCIOUS ORIENTED, NO NEURODEFICIET,

Investigations-Lab

ATTACHED WITH DISCHARGE SUMMARY

Investigations-Img

ATTACHED WITH DISCHARGED SUMMARY

Course In the Hospital

AS ABOVE MENTIONED HISTORY PT ADMITTED AND ACCORDINGLY INVESTIGATIONS

DONE, ECG SHOWS MULTIPLE VPCS, 2 D ECHO SHOWS EF 35%, DILATED AORTIC ROOT, TRIV MR,TR.ANT.SEPTAL INFERIOR HYPOKINESIA.SO HOLTER MONITOR DONE, REPORT SHOWES:MULTIPLE VENTRICULAR ECTOPICS, COUPLETS, TRIPLETS,, MULTIPLE BIGEMINY, TRIGEMINY NOTED, 2 EPISODES OF ILLSUSTAIINED VT, TREATED WITH INJ CORDARONE INFUSION AND TAB CORDARONE STARTED. BSL MONITOR, PT TREATED WITH DIURETICS, ANTIPLETLET, STATIN, ANTIANGINAL IRON SUPPLEMENT. PT REMAIN STABLE IN HOSPITAL STAY. AMBULATE WELL. HENCE DISCHARGE TO HOME WITH MEDICATION.

Treatment Given

INJ CORDARONE, TAB CORDARONE, TAB LASIX, TAB NITROCONTIN, TAB NOVASTAT, TAB CARDIVAS CR, TAB CARDACE, TAB ATO Z, TAB OROFER XT, TAB BRILLINTA, TAB ECOSPRIN,

Condition at Discharge

HEMODYNAMICALLY STABLE.

Treatment advice on Discharge

TAB CORDARONE 200MG	1----1-----1 X 1 DAY – Sunday
THEN	1----0-----1 X 3DAYS – Monday to Wednesday
THEN	1-----0----0 – Thursday onwards
TAB CEFTUM 500 MG	1-----0----1 X 5 DAYS – Sunday to Thursday
TAB BRILLINTA 90MG	1-----0----1
TAB ECOSPRIN 75MG	0-----1----0
TAB LASIX 40 MG	1-----0----0
TAB NITROCONTINE 2.6 MG	1-----1----0
TAB NOVASTAT 40 MG	0-----0----1
TAB CARDIVAS CR 10 MG	1-----0----0
TAB VALZAR 40	0-----0----1 – Stopped on Monday – 22 Apr.
TAB URIMAX D 0.4 MG	0-----0----1
TAB A TO Z	0-----1----0
TAB OROFER XT	0-----1----0
LIQ PARAFIN	2-----2----2 X 4 DAYS
NASOMIST SALINE	3---3----3----3 X 4 DAYS
CONTINUE TILL FOLLOW UP	

Diet Recommendation

DIABETIC DIET

Special Instructions

AS PER ADVISE

Follow up

FOLLOW UP WITH DR GAWARE SIR IN OPD AFTER 1WEEK WITH PRIOR APPOINTMENT.

Contact in Emergency

If you devlop any of the following complaint, please contact immediately.

1. Chest Pain / Discomfort / Heavyness in Chest.

2. Palpitation.

3. Giddiness.

4. Vomiting, Loose Motion.

5. Breathlessness.

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Signature of RMO Mayuri
Dr. Mayuri

Signature of Clinician Incharge

Entered By ANJALLEE

Dr. Prashant S.
2013062308